

Asthma Concerns: A Comparison of Somali Mothers and Fathers with Affected Preschoolers

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TRiO
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BACKGROUND

- ✓ Minnesota is home to the largest Somali population in the United States.
- ✓ However, little is known about the asthma-related concerns of Somali parents caring for young children with asthma or breathing difficulties.
- ✓ There are few culturally relevant resources available for Somali families of preschool children with asthma

PURPOSE

- ✓ To identify asthma-related concerns of low-income Somali mothers and fathers of preschool children (ages 3-5) with asthma who are enrolled in the Head Start program in St. Paul, MN.
- ✓ To compare mothers' and fathers' perspectives about childhood asthma.
- ✓ To contribute to the development of a culturally appropriate web-based asthma resources program for Somali parents of pre-school children**.**

RESEARCH QUESTIONS

- ✓ What are the types of asthma-related concerns that Somali parents of preschool children (ages 3-5) identify?
- ✓ What are the similarities and differences in the ways that Somali mothers versus fathers express asthma-related concerns?

RESEARCH DESIGN

- ✓ This secondary analysis of focus group findings with Somali parents of children with asthma was drawn from the Somali Head Start Asthma Project.
- ✓ This community-based, participatory action research project was conducted in partnership with Head Start health staff, interpreters and Somali parents of children enrolled in the Head Start Community Action Program of Ramsey County, in St. Paul, MN.

FINDINGS

Parents in both the mothers (M) and fathers (F) focus groups voiced concerns about their preschooler's asthma or breathing difficulties. Common concerns included: the Severity of Asthma, Medication (side effects and long term effects, unnecessary use of medications, medications causing asthma, medication before diagnosis and medication effectiveness), Recognition of Asthma (correct diagnosis by health care provider and recognition of symptoms by family members), Environmental Triggers of Asthma Symptoms, Asthma Disease Trajectory, and Asthma Management at School. Examples are provided for some of these concerns.

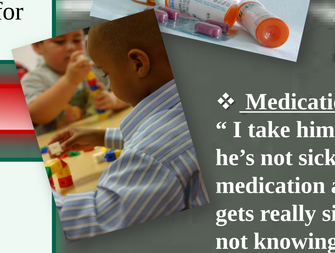
Severity of Asthma:

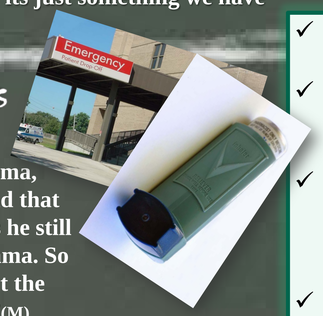
❖ **" I see it as a very very serious condition...if somebody cannot breathe, a child can't breathe and you know they could possibly die from that, so if the nebulizer or the machine is not working correctly- then you would go seek the help of a doctor, as soon as possible."(M)**



❖ **Long Term Effects:**
"...and when that medicine has a side effect… why does the doctor recommend to give to the child everyday, for those medicine, for twice a day 'causes if maybe if you give all the time maybe that one have more consequences in a long term for the kid ."(F)

❖ **Unnecessary Use of Medicine:**
"Yeah and we give him the medications everyday not knowin' you know, if the child actually needs those medications, so or if its just something we have to give ." (M)





❖ **Medication Use before Diagnosis:**
" I take him to the hospital. They tell me you know its not asthma, he's not sick and they gave me medications. And I give my child that medication and sometimes even after taking those medications he still gets really sick…they don't diagnose him necessarily with asthma. So not knowing you know, whether the child has asthma and what the medications are doing for the child is sometimes a confusion ."(M)

❖ **Correct Diagnosis by Heath Care Providers:**
" My child has been coughing for the past two years or so and every time I take him to the hospital they wouldn't say that he has asthma or a breathing difficulty they would rather say its OK you know its just a pneumonia...and he'll be fine..."(M)



❖ **Recognition of Symptoms by Parents:**
" when he's sleeping it looks like you know he's having difficulty breathing. So I would have to put pillows under him sometimes, you know to make sure he's breathing OK- a but at the same time its not easy for me to whether he's breathing, if he's having a difficulty breathing or if its just, you know, something else?"(M)



❖ **Environmental Triggers of Asthma Symptoms:**
" When my son comes to school sometimes the school people they will take the children out – you know during the school time, they allow they children to play outside but my son the time he outside- you know- the asthma is, there is asthma attacks...so, I would like them to stay indoors and not be allowed to play outside."(F)

METHODOLOGY

- ✓ Eligible Head Start Somali parents of preschool children (ages 3-5) who met asthma or breathing difficulty criteria were informed by Head Start nurses about the opportunity to participate in the study. Interested parents received a packet of information (in English and Somali) about the project and consent forms to review prior to the focus group.
- ✓ Separate mother and father focus groups were held on the same evening at a Head Start Center. Transportation, child care, and a light supper were provided. Each parent participant received a Target gift card. 5 mothers and 4 fathers participated in the focus group sessions.
- ✓ Each focus group session was led by a project investigator and a Head Start nurse co-facilitator along with an interpreter. Informed consent was obtained, prior to the focus group sessions which were audio recorded. Facilitators held a debriefing session after each group concluded.

ANALYSIS

- ✓ First, audiotaped focus group recordings were transcribed verbatim, and reviewed for accuracy.
- ✓ A working definition of asthma-related concern was created to guide the analysis of the participants' responses by two independent raters on the research team.
- ✓ Descriptive content analysis was used to identify and categorize asthma- related concerns in both the mothers' and fathers' group interviews. Concerns were then compared between the two groups for similarities and differences, and common themes were identified.
- ✓ The trustworthiness of the data was determined by checking the accuracy of the transcription and having two independent raters code the data and come to consensus on the identification of the types of asthma-related concerns.

CONCLUSIONS

- ✓ Although concerns were similar between mothers and fathers, two distinct lenses were evident in the way parents talked about asthma:
  - ➢ Mothers had a care centered theme focused on improving the care and condition of their child.
  - ➢ Fathers had an explanatory theme that was more question–specific and answer driven than care centered.

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Parents, teachers and staff at the Head Start Programs of the Community Action Partnership of Ramsey and Washington Counties. (Photos courtesy of : Anissa Thompson, Dewayne Flowers, Chiacat, Pawel Grawbowski and Dawnball1.)